MEDICAL-LEGAL ADVOCACY SCREENING QUESTIONNAIRE: EVALUATING EFFICACY

Bryan M. Polkey, McNair Scholar • Eileen Weber, DNP, JD, PHN, RN • School of Nursing • University of Minnesota

Significance

- One in six low-income families has at least one legal problem negatively affecting their health (Legal Services Corporation [LSC], 2009).
- Only one in five of these problems are addressed with the help of an attorney (LSC, 2009).
- Effective screening tools for healthcare legal partnerships (HLPs) are needed to identify those likely to use legal advocacy.

Background

- HLPs add legal advocacy to healthcare delivery to reduce health-harming social determinants.
- Formal HLPs began in 1993 to address the needs of asthmatic children living in substandard housing (Medical-Legal Partnership Boston, 2015).
- Currently 276 partnerships in 36 states provide services ranging from consultation to full representation (National Center for Medical-Legal Partnership, 2015).
- Patients are screened in the areas of Income, Housing, Education, Legal Status (immigration), and Personal Stability (I-HELP; National Center for Medical-Legal Partnership, 2015).
- Family Advocates of Central Massachusetts developed the Medical-Legal Advocacy Screening Questionnaire (MASQ) and determined it more effectively screened families in pediatric and family practice settings for legal service referral than the clinical interview alone (Keller, Jones, Savageau, & Cashman, 2008).

I-HELP® Areas



Image from the National Center for Medical-Legal Partnership

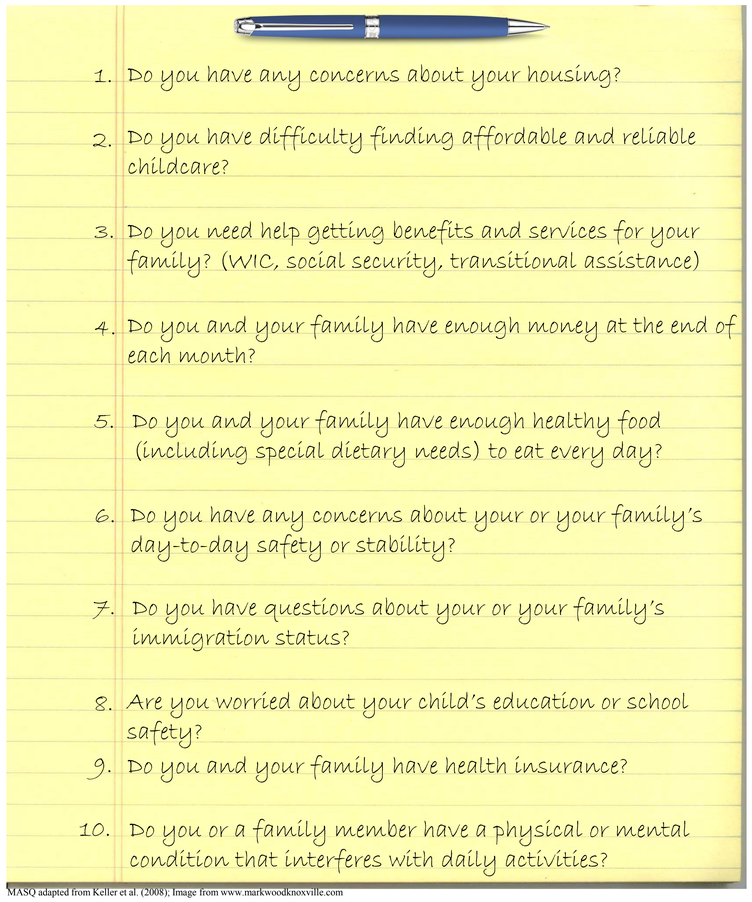
Study Aims

- **Primary Aim**: Determine if the MASQ is able to screen adults in primary care settings for referral to legal services more effectively than the clinical interview alone.
- **Secondary Aims**: Examine the MASQ as a needs assessment tool in establishing a healthcare legal partnership.

  Compare provider legal needs screening at a long standing healthcare legal partnership to a clinic with no healthcare legal partnership.

Medical-Legal Advocacy Screening Questionnaire (MASQ)

- 10-item screening tool scored using a 4-point Likert scale (No, Not Really, Sort of, Yes).
- Seventh grade reading level (Flesch-Kincaid Index 7.1).
- Four topic areas: Housing (1 question), Financial Stability (4 questions), Dignity and Safety (2 questions), and Access to Services (3 questions).



1. Do you have any concerns about your housing?
2. Do you have difficulty finding affordable and reliable childcare?
3. Do you need help getting benefits and services for your family? (WIC, social security, transitional assistance)
4. Do you and your family have enough money at the end of each month?
5. Do you and your family have enough healthy food (including special dietary needs) to eat every day?
6. Do you have any concerns about your or your family's day-to-day safety or stability?
7. Do you have questions about your or your family's immigration status?
8. Are you worried about your child's education or school safety?
9. Do you and your family have health insurance?
10. Do you or a family member have a physical or mental condition that interferes with daily activities?

MASQ adapted from Keller et al. (2008); Image from www.markwoodknoxville.com



Image from pinterest.com

Study Design

- **Design:** Cross-sectional, descriptive study.
- **Participant Goal:** 300 voluntary adult patients at participating clinic sites.
- **Setting:** (A) Adult, primary care nurse practitioner clinic with no established healthcare legal partnership and (B) comprehensive, primary care, federally-qualified healthcare center with a long standing healthcare legal partnership.
- **Methods:** Participants anonymously complete the MASQ (available in English and Spanish) and indicate a request for referral to legal services. After the clinical interview, without seeing the MASQ results, providers are asked if the participant should be referred to legal services.
- **Data Analysis:** The MASQ results and provider responses will be compared to the indicated referral response to calculate the MASQ sensitivity, specificity and positive predictive value. Differences in clinical site demographics will be compares among participants with positive and negative MASQ scores.

Study Timeline

April – June 2015: Study Development

June – August 2015: Field MASQ in clinic A

July – September 2015: Field MASQ in clinic B

September 2015: Data analysis and reporting

References

Keller, D., Jones, N., Savageau, J. A., & Cashman, S. B. (2008). Development of a brief questionnaire to identify families in need of legal advocacy to improve child health. *Ambulatory Pediatrics*, *8*(4), 266-269.

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Acknowledgments

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